



PRACTO- CARE AI



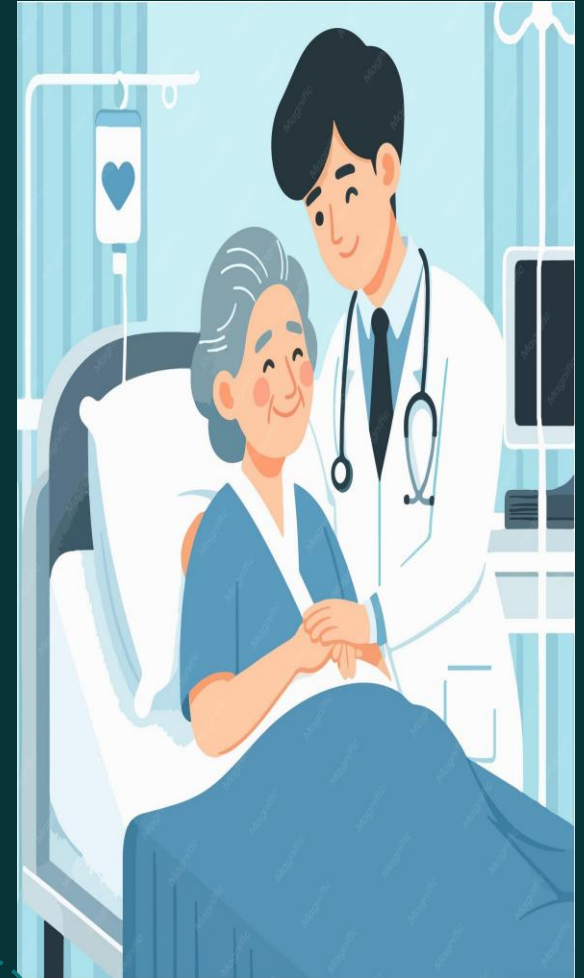
PRODUCT TEARDOWN



Care AI gets you to the right specialist but doesn't tell you why, doesn't prepare you for the visit, and doesn't acknowledge when it's uncertain. In healthcare, a recommendation you can't understand is a recommendation you can't trust.

MANAV AGARWAL

Live Link : [CareLogic](#) — by Care AI



PRACTO- INDIA'S DOMINANT HEALTHCARE MARKETPLACE

400M+

Cumulative users

50M+ patients served FY25

500K+

Doctors onboarded

640+ cities
20000+ AI calls/chats daily

₹234 Cr

₹ Revenue FY25

₹15 Cr

EBDITA – first ever profit

BUSINESS MODEL TENSION

Practo earns from listing fees and consultation commissions, creating an incentive to **maximize appointment bookings** rather than prioritize the best **doctor-patient match or care quality**.

1:834

Doctor-patient ratio nationally, but as poor as 1:11,082 in rural areas

30-40%

patients self-refer to specialists, platforms reinforce this

\$107B

patients self-refer to specialists, platforms reinforce this



COMPETITIVE POSITIONING

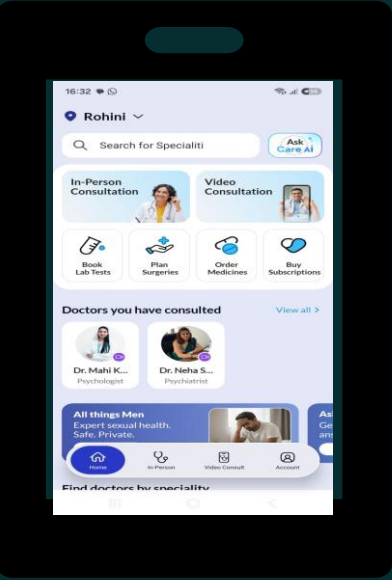
Unlike Apollo's closed hospital ecosystem and 1mg's pharmacy-first model, Practo operates the largest open healthcare marketplace. Yet its doctor routing remains opaque, creating an opportunity for transparent, explainable AI recommendations at scale.

Supply side is solved but **navigation** still remains an enigma. Let's investigate what happens when patients try to navigate

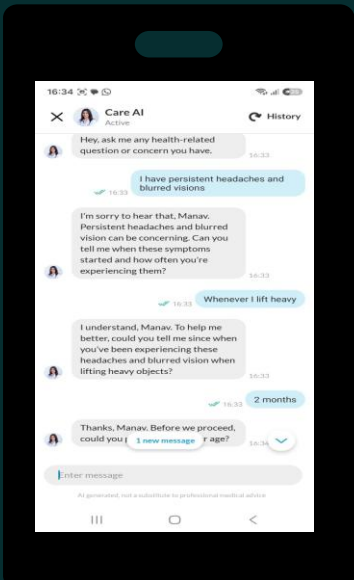
CARE AI DECIDES FOR YOU BUT NEVER TELLS YOU WHY

CARE AI USER JOURNEY- TESTED FIRHAND

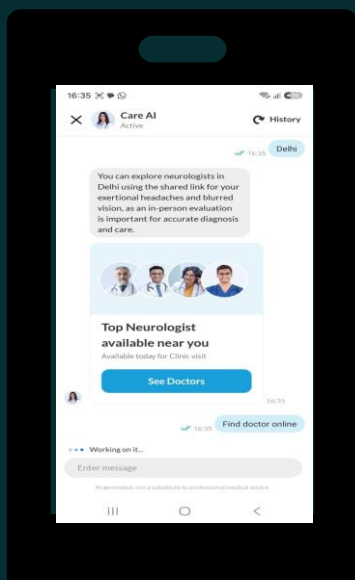
PATIENT VOICES + DATA EVIDENCE



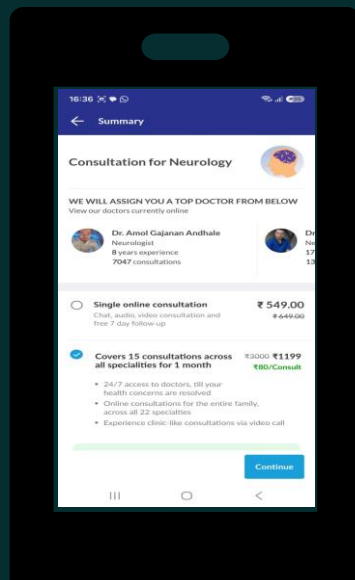
Step 1: Symptom intake
Empathetic, structured



Step 2: Specialist routing
Gets it right



Step 3 Why this specialist
Gets it right



Step 4 Why this doctor
Assigned, no transparency

82 of 1,046 negative reviews (~8%) cite **opaque doctor assignment** as a core complain

There's no choice of doctors. They assign them arbitrarily."

★ · 8 likes

Source: Play Store

"We will never know which doctor will talk to you."

★ · 3 likes

Source: Play Store

I asked for Piles doctor, was connected with a general physician."

★ · 2 likes

Source: Reddit

*Replaced patient guessing with AI guessing. The anxiety didn't go away - **it moved.**"*

But how big is the problem actually? -----> **Let's find out**

40-50% OF SELF-REFERRALS HIT THE WRONG SPECIALIST

OPACITY ISN'T THE FIX

STATS WORTH LOOKING AT

40-50%

Misreferral rate in self-referring patients

SOURCE: Indian OPD audit data

25-40%

Referral leakage even when GPs refer correctly

₹325 → ₹650+

Misrouting doubles current spend and compounds to time loss

20000+ AI Calls

Scope of opacity on Practo

DETERMINING ROOT CAUSE USING THE ICEBERG MODEL

Opaque assignment

No reasoning --> No choice --> Low Trust

Surface

PRACTO EARNS PER CONSULTATION

Practo's revenue increases with every successful consultation, making completed bookings the platform's primary business objective

INCENTIVISED TO MAXIMISE BOOKINGS

Success is measured by booking volume, encouraging recommendations that increase conversions rather than optimize doctor-patient match quality.

FAST OPAQUE ASSIGNMENT > SLOW TRANSPARENT CHOICE

Explaining recommendations introduces friction, so faster black-box suggestions often outperform transparent but slower decision-making experiences.

PATIENT BOOKS WITHOUT UNDERSTANDING

Patients receive recommendations but rarely understand why a doctor was chosen, reducing confidence in the platform's decision.

LOW TRUST ----> CHURN

When users can't trust recommendations, they're more likely to abandon the platform or seek advice elsewhere for future consultations.

Care AI solves the routing accuracy problem but lacks transparency----> Patients book once, don't trust the experience and **hardly come back.**

SAME AI ROUTING. COMPLETE TRANSPARENCY. PATIENTS DECIDE

Hypothesis A- Transparent recommendation engine

Adds reasoning layer to existing AI · Solves trust at the moment of decision · Directly reduces booking anxiety



Hypothesis B- Post-Consultation Validation Loop

Improves platform algorithm but adds zero value for the patient in the moment of decision

BEFORE CARE AI

Symptom intake

Empathetic, structured — this works

Specialist recommendation

“Here are neurologists” — no reasoning

Ruled-out alternatives

Nothing shown — patient left wondering

Doctor matching

Auto-assigned, no criteria visible

Social proof

Generic “4.3 stars” — not condition-specific

Consultation prep

Nothing, patient walks in blind

AFTER CARELOGIC BY CARE AI

Symptom intake

Empathetic, structured — this works

Specialist + Reasoning

Headache + vision changes → neurological origin, not eye problem

Ruled-out shown

Ruled out: Ophthalmologist — vision issue is secondary

3 doctors with visible criteria

Experience, similar-symptom patients, condition rating”

Condition-relevant proof

Headache patients rated Dr. Sharma 4.6/5 ”

Consultation prep checklist

Mention: headache duration, vision timing, medications

ARCHITECTURAL SYSTEM DESIGN

System Intake-
existing care AI



Triage + reasoning
Groq API -----> structured JSON output



Specialist + explanation
Reasoning +ruled-out alternative surfaced

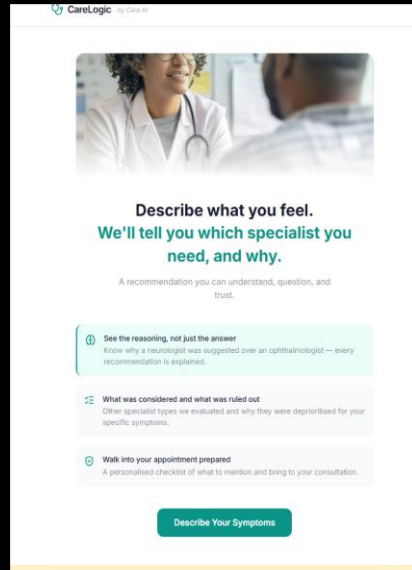


Doctor match + criteria
Patient chooses from 3 options with visible scoring

The AI does the same work. The patient **sees the thinking** to make an **informed decision**

I BUILT IT- TRY IT

Screen 1: Welcome page



CareLogic

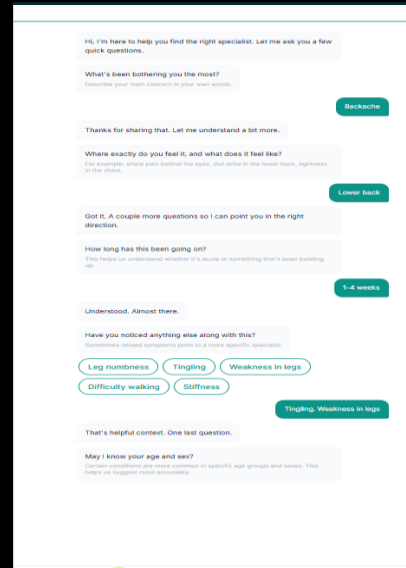
Describe what you feel.
We'll tell you which specialist you need, and why.

A recommendation you can understand, question, and trust.

- See the reasoning, not just the answer**
Know why a neurologist was suggested over an ophthalmologist — every recommendation is explained.
- What was considered and what was ruled out**
Other specialist types are evaluated and why they were deprioritized for your specific symptoms.
- Walk into your appointment prepared**
A personalized checklist of what to mention and bring to your consultation.

[Describe Your Symptoms](#)

Screen 2: Symptom intake



Hi, I'm here to help you find the right specialist. Let me ask you a few quick questions.

What's been bothering you the most?
Describe your main concern in your own words.

[Backache](#)

Thanks for sharing that. Let me understand a bit more.

Where exactly do you feel it, and what does it feel like?
For example: strong pain behind my eyes, dull ache in the lower back, tightness in the chest.

[Lower back](#)

Got it. A couple more questions so I can point you in the right direction.

How long has this been going on?
This helps us understand whether it's acute or something that's been building up.

[1-4 weeks](#)

Understood. Almost there.

Have you noticed anything else along with this?
Remember: related symptoms point to a more specific specialist.

[Leg numbness](#) [Tingling](#) [Weakness in legs](#)

[Difficulty walking](#) [Stiffness](#)

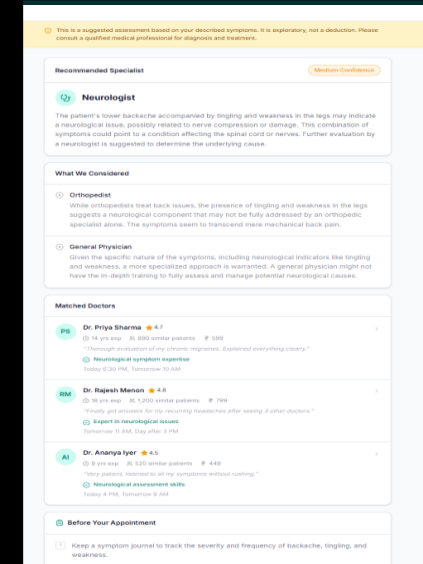
[Tingling, Weakness in legs](#)

That's helpful context. One last question.

May I know your age and sex?
Certain conditions are more common in specific age groups and sexes. This helps us suggest more accurately.

Patient describes backache + numbness. AI asks structured follow-ups.

Screen 3: Diagnosis and routing



This is a suggested assessment based on your described symptoms. It is exploratory, not a deduction. Please consult a qualified medical professional for diagnosis and treatment.

Recommended Specialist [Medium Confidence](#)

Neurologist

The patient's lower backache accompanied by tingling and weakness in the legs may indicate a neurological issue, possibly related to nerve compression or damage. This combination of symptoms could point to a condition affecting the spinal cord or nerves. Further evaluation by a neurologist is suggested to determine the underlying cause.

What We Considered

- Orthopedist**
While orthopedists treat back issues, the presence of tingling and weakness in the legs suggests a neurological component that may not be fully addressed by an orthopedic specialist alone. The symptoms seem to transcend mere mechanical back pain.
- General Physician**
Given the specific nature of the symptoms, including neurological indicators like tingling and weakness, a more specialized approach is warranted. A general physician might not have the in-depth training to fully assess and manage potential neurological causes.

Matched Doctors

- Dr. Priya Sharma** [4.7](#)
14 yrs exp. 10,000+ patient reviews. ["Thorough evaluation of the chronic symptoms. Explained everything clearly."](#)
[Neurological symptom expertise](#)
Today 10:30 PM, Tomorrow 10:30 AM
- Dr. Rajesh Menon** [4.8](#)
16 yrs exp. 15,000+ patient reviews. ["Really got across for my recurring headaches after seeing 3 other doctors."](#)
[Expert in neurological issues](#)
Tomorrow 11 AM, Day after 10 AM
- Dr. Ananya Iyer** [4.5](#)
8 yrs exp. 8,000+ patient reviews. ["Very patient, listened to all the symptoms without rushing."](#)
[Neurologist at assessment skills](#)
Today 4 PM, Tomorrow 9 AM

Before Your Appointment

- Keep a symptom journal to track the severity and frequency of backache, tingling, and weakness.

Neurologist recommended. Ruled out: Orthopedist and GP which is secondary.

3 doctors get recommended with visible scoring

LIVE PROTOTYPE ON VERCEL: [CareLogic](#) — by Care AI

MEASURING TRUST, NOT JUST TRANSACTIONS

★
NORTH STAR METRIC

Recommendation-to-booking conversation rate

% who see transparent recommendation → complete booking

Tied to---> revenue per user
Isolatable via A/B test. Real-time measurable

DRIVERS MOVING NORTH STAR METRIC

Reasoning Engagement

tap rate on "why not" + dwell time on reasoning card

-- Feature Adoption

Active doctor selection

% who compare doctors vs tap first option

--- Match Quality

Prep checklist completion

Checklist renders as interactive checkboxes, impression fires on viewport entry, each tick fires an event.
Completion = items checked ÷ items shown.

GUARDRAILS

Doctor availability

Slot fill must not hit 100%

--- Supply health

Red Flag detection

Emergency symptoms → immediate flag

---Patient Safety

Refund/ compliant rate

Must decrease as transparency increases

----Operational cost

~₹23 Cr
annual impact

20K daily AI calls × 10% conversion lift × ₹325 avg fee. Conservative, A/B testable. All inputs from Practo's own data.

WHAT COULD GO WRONG — AND HOW WE HANDLE IT.



RISK 1: Wrong reasoning, false confidence

AI explains confidently why you need a neurologist, but it's actually something more urgent. Authoritative tone delays emergency care.

MITIGATION

Suggestive tonality throughout — *"suggests," "may indicate," "commonly associated with."* Never declarative. Disclaimer above every output: *"This is a suggested assessment. It is exploratory, not a deduction."*



RISK 2: Regulatory exposure

Does showing clinical reasoning make Practo liable for medical advice under Telemedicine Practice Guidelines 2020?

MITIGATION

Key distinction: **surfacing reasoning ≠ diagnosing**. Low-risk transparency means showing what the AI considered, not prescribing treatment. Suggestive language is the legal boundary. Would need legal review before shipping at Practo's scale.



RISK 3: Patient overrides AI correctly

Patient reads "ruled out ophthalmologist" but actually has an eye problem. Transparency makes them dismiss the right specialist.

MITIGATION

Fallback built in: *"If this doesn't feel right, a general physician can evaluate and refer you appropriately."* Never close the door on alternatives. Acknowledge uncertainty as a design principle.